

Skin and Soft Tissue Infection

Patient Group Direction, version 002, issued 7 September 2026. Three arms: flucloxacillin, clarithromycin, doxycycline

Scope of this PGD

This PGD authorises the supply of an oral antibiotic for mild to moderate skin and soft tissue infection, including impetigo, folliculitis, infected eczema, infected wounds and cellulitis, in patients aged 2 years and over.

- **Flucloxacillin is first line. Clarithromycin and doxycycline may be used only where the patient has a penicillin allergy or flucloxacillin is otherwise unsuitable, and the reason must be recorded.**
- **Every observation in Appendix 1 must be measured and recorded before any supply. If any threshold is breached, refer.**
- Where cellulitis is treated, mark the skin margins and arrange review at 48 hours.
- Bite wounds are not covered by this PGD. Use the Minor Wound Care PGD.

Concurrent supply for infected eczema

Where eczema is complicated by MILD, LOCALISED secondary bacterial infection, a patient may receive an oral antibiotic under this PGD and a topical corticosteroid under the Eczema and Dermatitis PGD at the same consultation, both recorded in one consultation record. Where the infection is not mild and localised, or any exclusion here applies, refer and supply neither.

Arm 1 — Flucloxacillin (first line)

Patient Group Direction for the supply of flucloxacillin for impetigo, folliculitis, infected eczema, infected wounds and cellulitis, in patients aged 2 years and over.

Healthcare professionals covered, and training

- Pharmacist registered and practising with the GPhC.
- Pharmacy technician registered and practising with the GPhC. None of these medicines is a controlled drug, so registered pharmacy technicians may lawfully supply under this PGD (Human Medicines Regulations 2012 as amended, 26 June 2024).
- Must have completed training relevant to this condition, documented and overseen by the Get Real Health team.
- Must be familiar with the SPC for the product supplied, and with BNF advice.
- Must be competent in measuring and recording the observations in Appendix 1, and in recognising the necrotising fasciitis features listed there.
- Must previously have used PGDs to supply medication.
- Must work in compliance with the SOPs of their own employer and practise only within the bounds of their own competence.

The PGD

Indication	Mild to moderate skin and soft tissue infection: impetigo, folliculitis, infected eczema, infected wounds, and cellulitis, in patients aged 2 years and over. First-line agent.
Inclusion criteria	• Aged 2 years and over. • Localised skin or soft tissue infection with signs of bacterial infection: erythema, warmth, swelling, tenderness or purulent discharge. • All observations within the thresholds in Appendix 1. • Where cellulitis is being treated, the affected area is limited, the skin margins have been marked, and the patient can attend for review at 48 hours. • Valid informed consent given (from a person with parental responsibility where the patient is a child). • No exclusion criterion present.
Exclusion criteria	Refer, do not supply, where any of the following is present. • ANY feature suggesting necrotising fasciitis: pain out of proportion to appearance, rapidly advancing erythema, crepitus, skin necrosis, bullae, or dusky discoloration. Arrange emergency assessment, not a routine referral. • Any observation outside the thresholds in Appendix 1, or any sign of systemic illness or sepsis. • Abscess requiring drainage, or an infected wound needing surgical review. • Facial or periorbital cellulitis, or cellulitis of the hand. • Animal or human bite. Refer; these need co-amoxiclav, which is not covered here. • Suspected osteomyelitis or septic arthritis, or infection over a joint or tendon. • Immunosuppression of any kind. • Diabetic foot infection. • An antibiotic already taken for this episode. • Known penicillin or beta-lactam allergy. Use Arm 2 or Arm 3. • History of flucloxacillin-associated jaundice or hepatic dysfunction. • Under 2 years of age. • Severe renal impairment (creatinine clearance below 10 mL/min).
Cautions	• Flucloxacillin may be supplied in pregnancy and breastfeeding where clinically indicated. This aligns with the Wound Care PGD.

	• Hepatic reactions may occur up to two months after treatment; advise the patient to report jaundice or dark urine. • Advise that any rash should stop the course and prompt review.
Actions if excluded or declines	Explain why treatment cannot be supplied. Where any necrotising fasciitis feature is present, arrange EMERGENCY assessment. Where an observation threshold is breached or an abscess needs drainage, arrange same-day assessment. Document the advice and the decision, and inform the GP where the reason for exclusion is a new clinical finding.

The medicine

Name, form and strength	Flucloxacillin 500mg capsules; flucloxacillin 250mg/5mL oral suspension for children unable to swallow capsules.
Legal category	POM.
Route and method	Oral. Take on an empty stomach, one hour before or two hours after food, for optimal absorption.
Dose and frequency	Adults and children 10 years and over: 500mg four times daily. Children 2 to 9 years: 250mg four times daily (10mL of the 250mg/5mL suspension). Duration 5 days for uncomplicated infection; 7 days for cellulitis.
Quantity to be supplied	Capsules, 500mg four times daily: 20 capsules for 5 days, 28 capsules for 7 days. Suspension, 250mg four times daily: 100mL for 5 days, 140mL for 7 days. Supply the whole course. Do not split.
Maximum treatment period	7 days. One course per episode. A second course is not authorised; refer.
Adverse effects	Very common: nausea, diarrhoea. Common: rash, vomiting, abdominal discomfort. Rare but serious: anaphylaxis, cholestatic jaundice and hepatitis (may be delayed up to two months), Clostridioides difficile infection, acute generalised exanthematous pustulosis.
Yellow Card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP.
Records to be kept	• That valid informed consent was given. • Patient name, address, date of birth, and the GP with whom they are registered. • The observations from Appendix 1, and the site and extent of the infection. • Where cellulitis is treated, that the skin margins were marked and the patient advised of the 48-hour review. • Name and registration number of the healthcare professional supplying. • Name of the medicine, date of supply, dose, form, route and quantity supplied. • Batch number and expiry date. • For the second- and third-line arms, the reason flucloxacillin was unsuitable. • Advice given, including advice given if excluded or declining treatment. • Details of any adverse drug reactions and the actions taken. • That the medicine was supplied under this PGD. Records signed, dated, legible and contemporaneous. Adults: 8 years. Under

	18s: until the 25th birthday.
Storage	Capsules below 25C. Reconstituted suspension: refrigerate and discard after 7 days.

Information for the patient

Written information	Supply the flucloxacillin patient information leaflet.
Counselling	• Take on an empty stomach, an hour before food or two hours after, and finish the course. • If you have cellulitis, we have marked the edge of the redness. If the redness spreads beyond that mark, seek help the same day. • Seek help THE SAME DAY if the pain becomes severe or out of proportion to how it looks, if the area spreads quickly, if the skin darkens or blisters, or if you develop fever, shivering or feel generally unwell. • Stop and seek urgent help if you develop a rash, wheeze, or swelling of the lips or tongue. • Report yellowing of the eyes or skin, or dark urine, even weeks after finishing. • Come back if you are no better in 2 to 3 days.
Follow-up	Review at 48 hours where cellulitis is being treated. Otherwise seek advice if not improving within 2 to 3 days, or sooner if any of the warning signs above develop.

Arm 2 — Clarithromycin (penicillin allergy)

Patient Group Direction for the supply of clarithromycin where flucloxacillin is unsuitable, in patients aged 2 years and over.

This arm may only be used where the patient has a penicillin allergy or flucloxacillin is otherwise unsuitable. It is not a first-line option.

Healthcare professionals covered, and training

- Pharmacist registered and practising with the GPhC.
- Pharmacy technician registered and practising with the GPhC. None of these medicines is a controlled drug, so registered pharmacy technicians may lawfully supply under this PGD (Human Medicines Regulations 2012 as amended, 26 June 2024).
- Must have completed training relevant to this condition, documented and overseen by the Get Real Health team.
- Must be familiar with the SPC for the product supplied, and with BNF advice.
- Must be competent in measuring and recording the observations in Appendix 1, and in recognising the necrotising fasciitis features listed there.
- Must previously have used PGDs to supply medication.
- Must work in compliance with the SOPs of their own employer and practise only within the bounds of their own competence.

The PGD

Indication	Mild to moderate skin and soft tissue infection, including cellulitis, where flucloxacillin is unsuitable, in patients aged 2 years and over.
Inclusion criteria	• PENICILLIN ALLERGY, OR FLUCLOXACILLIN OTHERWISE UNSUITABLE, with the reason recorded. • Aged 2 years and over. • Localised skin or soft tissue infection with signs of bacterial infection. • All observations within the thresholds in Appendix 1. • Where cellulitis is being treated, margins marked and 48-hour review arranged. • Valid informed consent given. • No exclusion criterion present.
Exclusion criteria	Refer, do not supply, where any of the following is present. • ANY feature suggesting necrotising fasciitis: pain out of proportion to appearance, rapidly advancing erythema, crepitus, skin necrosis, bullae, or dusky discoloration. Arrange emergency assessment, not a routine referral. • Any observation outside the thresholds in Appendix 1, or any sign of systemic illness or sepsis. • Abscess requiring drainage, or an infected wound needing surgical review. • Facial or periorbital cellulitis, or cellulitis of the hand. • Animal or human bite. Refer; these need co-amoxiclav, which is not covered here. • Suspected osteomyelitis or septic arthritis, or infection over a joint or tendon. • Immunosuppression of any kind. • Diabetic foot infection. • An antibiotic already taken for this episode. • Hypersensitivity to macrolides. • Known QT prolongation, or concurrent QT-prolonging medicines. • Concurrent ergot alkaloids, oral midazolam, lomitapide, astemizole, cisapride, domperidone, pimozide, terfenadine, ticagrelor, ivabradine, ranolazine, simvastatin or lovastatin. • Taking warfarin or a DOAC. Refer.

	- Severe hepatic impairment, or known electrolyte disturbance. - Pregnancy or breastfeeding. Refer.
Cautions	- Colchicine: risk of toxicity. Refer rather than supply. - Advise on dizziness and taste disturbance.
Actions if excluded or declines	As Arm 1.

The medicine

Name, form and strength	Clarithromycin 250mg tablets; clarithromycin 125mg/5mL and 250mg/5mL oral suspension.
Legal category	POM.
Route and method	Oral, with or without food.
Dose and frequency	Adults and children 12 years and over: 250mg twice daily. 500mg twice daily for more extensive infection. Children by weight, twice daily: 8 to 11kg, 62.5mg. 12 to 19kg, 125mg. 20 to 29kg, 187.5mg. 30 to 40kg, 250mg. A child under 12 weighing more than 40kg receives the adult dose of 250mg twice daily. A child weighing under 8kg is outside this PGD; refer. Duration 5 days; 7 days for cellulitis.
Quantity to be supplied	Tablets, 250mg twice daily: 10 tablets for 5 days, 14 for 7 days. Tablets, 500mg twice daily: 20 tablets for 5 days, 28 for 7 days. Suspension volumes, calculated from the dose and stated per band. Using 125mg/5mL for the two smallest bands and 250mg/5mL above: 8 to 11kg (62.5mg BD = 2.5mL BD of 125mg/5mL): 25mL for 5 days, 35mL for 7 days. 12 to 19kg (125mg BD = 5mL BD of 125mg/5mL): 50mL for 5 days, 70mL for 7 days. 20 to 29kg (187.5mg BD = 3.75mL BD of 250mg/5mL): 37.5mL for 5 days, 52.5mL for 7 days. 30 to 40kg (250mg BD = 5mL BD of 250mg/5mL): 50mL for 5 days, 70mL for 7 days. Adults, 250mg BD as suspension (5mL BD of 250mg/5mL): 50mL for 5 days, 70mL for 7 days.
Maximum treatment period	7 days. One course per episode; a second course is not authorised.
Adverse effects	Common: abdominal pain, nausea, diarrhoea, taste disturbance, headache. Uncommon: QT prolongation, hepatic dysfunction. Rare: C. difficile infection, Stevens-Johnson syndrome.
Yellow Card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP.
Records to be kept	- That valid informed consent was given. - Patient name, address, date of birth, and the GP with whom they are registered. - The observations from Appendix 1, and the site and extent of the infection. - Where cellulitis is treated, that the skin margins were marked and the patient advised of the 48-hour review.

	• Name and registration number of the healthcare professional supplying. • Name of the medicine, date of supply, dose, form, route and quantity supplied. • Batch number and expiry date. • For the second- and third-line arms, the reason flucloxacillin was unsuitable. • Advice given, including advice given if excluded or declining treatment. • Details of any adverse drug reactions and the actions taken. • That the medicine was supplied under this PGD. Records signed, dated, legible and contemporaneous. Adults: 8 years. Under 18s: until the 25th birthday.
Storage	Tablets below 25C. Reconstituted suspension per the SPC; do not refrigerate.

Information for the patient

Written information	Supply the clarithromycin patient information leaflet.
Counselling	• Take twice a day and finish the course. • Tell us or your GP before starting any new medicine. • If you have cellulitis, seek help the same day if the redness spreads beyond the marked edge. • Seek help the same day if pain becomes severe or out of proportion, the skin darkens or blisters, or you develop fever or feel generally unwell. • Come back if you are no better in 2 to 3 days.
Follow-up	As Arm 1. Refer if symptoms worsen or do not improve within 2 to 3 days.

Arm 3 — Doxycycline (penicillin allergy, 12 years and over)

Patient Group Direction for the supply of doxycycline 100mg capsules where flucloxacillin is unsuitable, in patients aged 12 years and over.

This arm may only be used where the patient has a penicillin allergy or flucloxacillin is otherwise unsuitable. It is not a first-line option.

Healthcare professionals covered, and training

- Pharmacist registered and practising with the GPhC.
- Pharmacy technician registered and practising with the GPhC. None of these medicines is a controlled drug, so registered pharmacy technicians may lawfully supply under this PGD (Human Medicines Regulations 2012 as amended, 26 June 2024).
- Must have completed training relevant to this condition, documented and overseen by the Get Real Health team.
- Must be familiar with the SPC for the product supplied, and with BNF advice.
- Must be competent in measuring and recording the observations in Appendix 1, and in recognising the necrotising fasciitis features listed there.
- Must previously have used PGDs to supply medication.
- Must work in compliance with the SOPs of their own employer and practise only within the bounds of their own competence.

The PGD

Indication	Mild to moderate skin and soft tissue infection, including cellulitis, where flucloxacillin is unsuitable, in patients aged 12 years and over.
Inclusion criteria	• PENICILLIN ALLERGY, OR FLUCLOXACILLIN OTHERWISE UNSUITABLE, with the reason recorded. • Aged 12 years and over. • Localised skin or soft tissue infection with signs of bacterial infection. • All observations within the thresholds in Appendix 1. • Where cellulitis is being treated, margins marked and 48-hour review arranged. • Valid informed consent given. • No exclusion criterion present.
Exclusion criteria	Refer, do not supply, where any of the following is present. • ANY feature suggesting necrotising fasciitis: pain out of proportion to appearance, rapidly advancing erythema, crepitus, skin necrosis, bullae, or dusky discoloration. Arrange emergency assessment, not a routine referral. • Any observation outside the thresholds in Appendix 1, or any sign of systemic illness or sepsis. • Abscess requiring drainage, or an infected wound needing surgical review. • Facial or periorbital cellulitis, or cellulitis of the hand. • Animal or human bite. Refer; these need co-amoxiclav, which is not covered here. • Suspected osteomyelitis or septic arthritis, or infection over a joint or tendon. • Immunosuppression of any kind. • Diabetic foot infection. • An antibiotic already taken for this episode. • Under 12 years of age. • Pregnancy or breastfeeding. Refer. • Hypersensitivity to doxycycline or other tetracyclines. • Severe hepatic impairment. • Myasthenia gravis, systemic lupus erythematosus, or porphyria. • Concurrent isotretinoin. Refer.

	• Taking warfarin. Doxycycline potentiates it; refer.
Cautions	• Photosensitivity: advise sun protection. • Separate from antacids, iron and dairy by at least 2 hours.
Actions if excluded or declines	As Arm 1.

The medicine

Name, form and strength	Doxycycline 100mg capsules.
Legal category	POM.
Route and method	Oral. Swallow whole with plenty of water, sitting or standing, and well before lying down.
Dose and frequency	200mg on day 1, then 100mg daily. 200mg daily throughout for more extensive infection. Duration 5 days; 7 days for cellulitis.
Quantity to be supplied	100mg daily regimen: 6 capsules for 5 days, 8 for 7 days. 200mg daily regimen: 10 capsules for 5 days, 14 for 7 days.
Maximum treatment period	7 days. One course per episode; a second course is not authorised.
Adverse effects	Common: nausea, diarrhoea, photosensitivity. Uncommon: oesophageal irritation or ulceration, rash, headache. Rare: C. difficile infection, hepatotoxicity, benign intracranial hypertension, Stevens-Johnson syndrome.
Yellow Card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP.
Records to be kept	• That valid informed consent was given. • Patient name, address, date of birth, and the GP with whom they are registered. • The observations from Appendix 1, and the site and extent of the infection. • Where cellulitis is treated, that the skin margins were marked and the patient advised of the 48-hour review. • Name and registration number of the healthcare professional supplying. • Name of the medicine, date of supply, dose, form, route and quantity supplied. • Batch number and expiry date. • For the second- and third-line arms, the reason flucloxacillin was unsuitable. • Advice given, including advice given if excluded or declining treatment. • Details of any adverse drug reactions and the actions taken. • That the medicine was supplied under this PGD. Records signed, dated, legible and contemporaneous. Adults: 8 years. Under 18s: until the 25th birthday.
Storage	Store below 25C.

Information for the patient

Written information	Supply the doxycycline patient information leaflet.
Counselling	• Take with plenty of water, sitting or standing, and do not lie down for

	30 minutes. ● Avoid antacids, iron and milk within 2 hours of a dose. ● You may burn more easily in the sun. ● If you have cellulitis, seek help the same day if the redness spreads beyond the marked edge. ● Seek help the same day if pain becomes severe or out of proportion, the skin darkens or blisters, or you develop fever or feel generally unwell. ● Come back if you are no better in 2 to 3 days.
Follow-up	As Arm 1.

Appendix 1 — Observations, and necrotising fasciitis features

Every observation must be measured and recorded before any supply. If ANY threshold is breached, do not supply: refer. These thresholds match the Acute Bacterial Bronchitis PGD so that the two documents agree.

Temperature	• REFER if 38C or above.
Pulse	• REFER if above 110 at rest.
Respiratory rate	• REFER if 22 or above.
Blood pressure	• REFER if systolic below 100.
Oxygen saturation	• REFER if SpO2 below 94% on air at rest.
Level of consciousness	• REFER on any new confusion or drowsiness.

Necrotising fasciitis — emergency, not a referral

Early necrotising fasciitis looks like cellulitis. Any one of these means emergency assessment now, not an antibiotic and a review appointment.

- Pain out of proportion to the appearance of the skin.
- Rapidly advancing erythema, spreading over hours rather than days.
- Crepitus, or gas in the tissues.
- Skin necrosis, bullae, or dusky or purplish discoloration.
- Anaesthesia over the affected skin.
- Systemic illness out of keeping with the appearance of the skin.

Cellulitis: marking and review

- Mark the edge of the erythema with a skin-safe pen before the patient leaves, and record that you have done so.
- Review at 48 hours. Spread beyond the mark means same-day referral, not a change of antibiotic.
- Facial, periorbital and hand cellulitis are excluded from this PGD entirely.

Authorisation

Version	v002
Supersedes	Version 001
Valid from date	7 September 2026
Expiry date	31 July 2027

Doctor	Name: Nitin Shori Job title: Medical Director, Get Real Health GMC: 6047293 Signed: N. Shori Date: 7 September 2026
Pharmacist	Name: Chris Pilkington Job title: Head Pharmacist, Get Real Health GPhC: 2046322 Signed: C. Pilkington

	Date: 7 September 2026
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Both authorising signatories reviewed this version together on 7 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs.

Adoption by the employing organisation

To be completed by the adopting pharmacy. These signatories are the adopting organisation own signatories and must not be pre-filled by Get Real Health.

Superintendent / clinical lead	Name:
	Job title and organisation:
	Signature:
	Date:
Professional group signatory	Name:
	Job title and organisation:
	Signature:
	Date:

Change history

Version	v002
Date	7 September 2026
Changes	• Full clinical review and reissue.